Department of Statistics

University of Rajshahi

Research Project On

"A Comparative study on treatment system of government and private hospitals in Munshiganj''

CHAPTER ONE

Research Perspective

1.1 Introduction

Bangladesh has a well-developed healthcare network that serves both rural and urban regions. A regulated and regulated system of finance, management, scope, and content for a specific group of people. The provision of services to people in order to improve their health is referred to as "healthcare services." these are given in specific places, such as homes, schools, businesses, public spaces, neighborhoods, hospitals, and clinics. There are 3,976 public-sector healthcare institutions and 975 privately managed hospitals and clinics.(Miah1*, 2, & Afrin3, 2020). Bangladesh faces unique challenges in providing healthcare to its large and diverse population. The government-run healthcare system, with its extensive network of hospitals and clinics, aims to serve as a safety net for the majority of the population, focusing on affordability and accessibility. On the other hand, the private healthcare sector has witnessed significant growth in recent years, offering advanced medical facilities, superior technology, and a higher standard of care, albeit at a cost.

This comparative study seeks to comprehensively evaluate and contrast the two healthcare systems, considering factors such as quality of care, cost, accessibility and the role each sector plays in shaping the healthcare landscape of Bangladesh. By shedding light on the strengths and weaknesses of both systems, this study aims to provide insights into the challenges, opportunities, and disparities within the healthcare sector. Additionally, it explores the potential policy changes and reforms that could lead to a more equitable, accessible, and efficient healthcare system in Bangladesh.

Ultimately, this study contributes to the ongoing discourse on healthcare reform in Bangladesh, with the overarching goal of improving healthcare outcomes and ensuring that all citizens, regardless of their economic status, have access to quality medical care. In doing so, it aims to enhance the overall health and well-being of the people of Bangladesh and create a more inclusive and sustainable healthcare system for the future.

1.2 Background of the Study

Healthcare is a fundamental aspect of human well-being, and access to quality medical services is a critical determinant of public health. In Munshiganj, a district in Bangladesh, the healthcare sector plays a pivotal role in ensuring the health and welfare of its residents. The healthcare landscape in Munshiganj, like in many parts of the world, is composed of a mix of government and private hospitals that provide medical services to the local population.

Government hospitals are typically funded and operated by the state and cater to a broad section of the population, aiming to provide essential healthcare services to all, regardless of their socio-economic status. On the other hand, private hospitals are privately owned and operated institutions that offer healthcare services in a competitive market, often requiring patients to pay for services either through out-of-pocket expenses or insurance.

Understanding the dynamics and differences between these two types of healthcare systems is crucial for policymakers, healthcare providers, and patients. This study seeks to provide a comprehensive comparative analysis of the treatment systems in government and private hospitals in Munshiganj, with a focus on various critical aspects.

One of the primary motivations for this study is the growing importance of healthcare accessibility, quality, and affordability. Munshiganj, like many regions in Bangladesh, faces challenges in delivering adequate healthcare services to its population, including issues related to the availability of healthcare facilities, quality of care provided, and the financial burden placed on patients and their families. These challenges are exacerbated by the dual presence of government and private hospitals, each with its own distinct characteristics and service delivery models.

By conducting a comparative analysis of these healthcare systems, this study aims to shed light on the strengths and weaknesses of government and private hospitals in Munshiganj. This research will not only benefit the local population by helping them make informed healthcare decisions but also provide insights that can inform healthcare policy decisions and strategies to improve the overall healthcare system in the district.

The findings from this study have the potential to guide policymakers in developing and implementing effective healthcare policies, which can enhance healthcare accessibility, improve the quality of care, and make healthcare services more affordable for all residents of Munshiganj. Furthermore, this research may serve as a model for similar studies in other regions facing similar healthcare challenges.

In summary, the comparative analysis of treatment systems in government and private hospitals in Munshiganj is a crucial study that addresses significant healthcare issues. It strives to inform healthcare policy decisions, empower patients, and ultimately contribute to the overall well-being of the community.

1.3 Objectives of the Study

This study is useful in a variety of ways. This study will provide light on the disparities in service quality between public and private hospitals. This will also give a better idea of which concerns require extra attention. As the government works to enhance healthcare quality, the objectives of the study are:

- To compare the treatment system of public and private hospitals has to claim patient satisfaction in Bangladesh.
- To analyze and compare the cost of healthcare services in government and private hospitals, including price transparency, out-of-pocket expenses, and insurance coverage.

By addressing these objectives, this study seeks to provide a comprehensive understanding of the healthcare landscape in Munshiganj, with a focus on the differences between government and private hospitals. These objectives will guide the research process and help generate valuable insights into the healthcare sector in the region.

CHAPTER TWO

Literature Review

This chapter discusses the major theoretical frameworks that have been used to fulfill this study's objectives. So, on our journey to know which service quality from public and private hospitals affects patient's satisfaction. We have gone through some papers that have bought about some key aspects of my research.

This chapter discusses the major theoretical frameworks that have been used to fulfill this study's objectives. So, on our journey to know which service quality from public and private hospitals affects patient's satisfaction. I have gone through some papers that have bought about some key aspects of my research.

2.1 Health care system

Healthcare is one of the most vital components of human life. Health facilities are used to diagnose, manage, and treat illnesses to meet this need. The provision of services to people in order to improve their health is referred to as "healthcare services".

A Study by (SaadAndaleeb, 2000) stated that Bangladesh's health care system is under attack from a number of problems that point to an impending crisis. With a growing population and little investment in the health care sector, service seems to have gone down a lot. There are two main strategic goals that must be met to avoid any major crisis: increasing access and improving quality. This is what people say. However, it's still not clear what people think about the country's health care system.

According to a report (Tashin) stated that, Bangladesh's healthcare system is decentralized. The number of doctors per 10,000 inhabitants is 3.06, which is low. A third of sanctioned healthcare employees are unemployed. Bangladesh's healthcare system is decentralized and largely for-profit. Affordability forces impoverished people to skip treatment or go into debt. This financial load may be reduced by increasing healthcare spending.

2.2 Public Health System

According to a study (Aagja & Garg., 2010) stated that the public hospital is being developed to assess the five dimensions of hospital service quality: admission, medical service, overall service and discharge process. As a diagnostic tool, the scale might be used to identify particular improvements that need to be made in the hospital's services as well as components of those

services that need to be changed. (Almomani, Al-Ghdabi, & Banyhamda., 2019) This study's goal was to assess the influence of health care quality on patient satisfaction. In Jordan, public hospitals were studied. A convenience sample of 400 patients was used in this study, of which 354 were valid for statistical purposes. The AMOS 20 program was used to evaluate the proposed structural equation model. Quality of health care explained 65.7 percent of the variance in patient happiness, demonstrating how quality of health care can affect patient satisfaction in public hospitals. Patient satisfaction was positively influenced by healthcare quality, so researchers made suggestions for public hospital top management.

2.3 Private Health System

According to a study from (HANNAH, 2021), the study's goal is to get a patient's perspective on the quality of care provided in Iranian private hospitals. By conducting regular audits of the hospital's services, it is possible to enhance the quality of care provided to patients and their families. A patient's level of perception is definitely influenced by the quality of the care they get. In order to improve quality, the hospital and its staff must work together. Authorities must recognize that service quality procedures must be implemented at this time.

2.4 Service Quality

According to (Miah, & Afrin, 2020)" The researchers performed a survey of 376 patients and determined that both sectors are comparatively superior in terms of cost, physician's costs and perception, emergency treatment and patient waiting times, physician assurance and competence, empathy of doctors and nurses, and medication availability. Although their major conclusion reveals that both sectors are failing to meet global quality standards, they also highlight that the government must act quickly to provide support. In their statement, they stressed that the government must guarantee that quality services are offered in all parts of the nation; especially in rural areas where people are either unaware of or unable to adequately communicate their health concerns.

(Alumran, Hanadi Almutawa, & Khalid, 2020) They found that Saudi Arabia's healthcare system has undergone major changes in recent years. This study compared the quality of public and private hospitals in Eastern Saudi Arabia. It found that patients at private hospitals perceived a higher level of quality in the health care services ($t = 3.390$, $p < 0.01$). They conclude that further research on the financial and leadership dimensions of health care quality will contribute to

improve planning for health care services. (Yousapronpaiboon, D.B.A., & William C. Johnson, 2013)Suggest that, several service quality characteristics in private hospitals were not delivered as effectively as in public hospitals, and both groups may and should improve.

A study has found that,(Asrafi & Sabbit, 2015)When it comes to integrating e-health solutions, the private hospital is far ahead of the public hospital in terms of efficiency. The private hospital features advanced information and communications technology infrastructure, as well as a specialized IT department staffed by highly trained professionals who have previously developed, installed, and used e-health solutions. This research compares public and private hospitals in terms of service quality. The SERVQUAL instrument seems to be a good tool for measuring and monitoring hospital service quality. The SERVQUAL instrument seems to be a valuable tool for measuring and monitoring service quality in hospitals, allowing employees to identify areas for improvement from the customers' viewpoint.

2.5 Cost of treatment

This variable refers to the cost of healthcare services that patients must pay as well as their level of satisfaction. According to the findings of this research (Rahman1, Al-Amin, & Mahfuzur, 2020), private health care facilities are more efficient. The expense of obtaining health care from private institutions is too expensive for those who are less fortunate than themselves. Consequently, the government should make certain that public health institutions are both efficient and fair in their operations. When it comes to efficiency, private health care facilities beat public health care facilities by a wide margin. Ultimately, they discovered that, if the government wants to minimize the cost of public health facilities, it must ensure that the facilities are transparent.

The following is what happened as a consequence of their research: Government policy should place a higher priority on the availability of doctors, personnel, and equipment than on the creation of new medical facilities.

"According to the findings of (Priyanka Saksena1;Ke Xu1; Riku Elovainio1;Jean Perrot1;, 2010) poll, they found that the use of private health care facilities is not exclusive to the rich. Price competition may not be the most essential consideration for patients in many countries. Consultation costs for inpatient and outpatient sessions at public institutions account for just 10% of total out-of-pocket expenses. However, this may be at the expense of the private sector, which varies widely from country to country in terms of its significance. Everyone's access to services

and treatments must be improved as a result of policymakers' actions, according to the World Health Organization."

According to (Begum, Said, & Hossain., 2021) they found that, Bangladesh's inadequate governmental and commercial health institutions have deteriorated healthcare quality. Quality at a reasonable price may be fulfilled, but not to the patient's satisfaction and general well-being. This research compares public and private health care costs and satisfaction. In a public-private comparison, absolute satisfaction cost 180.30$ for public (4.3%) and 766.16$ for private hospitals (30.2 percent patients).

2.7 Responsiveness of doctor

According to (Sanjay Basu1, Andrews1, & Kishore, 2012)Comparing public and commercial healthcare systems in low- and middle-income nations revealed strengths and deficiencies in both sectors for each of the WHO's six primary concerns. The research found that private healthcare systems lacked publicly accessible data, provided poor service, and were less responsive to patients' concerns. The results were inconsistent in several cases, and the assessment failed to account for a variety of potential moderating factors.

2.8 Empathy of Doctor

The degree of worry that doctors and nurses have for their patients is measured by their empathy. According to an article (Coarasa, DasJ, & Hammer, 2014) , every doctor employs treatments that are unneeded and even dangerous. One research looked on the ways in which Bangkok's doctor’s deal with worry. 1.5 minutes (public sector, metropolitan India) to 8 minutes (private sector, rural India) for local recruiters who present the same problem to suppliers (private sector, Kenya). Patient care is being provided to less than 5 percent of those who need it most.

In their study, the authors discovered that there was no difference in treatment accuracy between public and private providers.

2.9 Patient Satisfaction

Patients' satisfaction with doctors' services is measured here. According to the findings of (Singh, 2013), private hospitals provide better patient care than state institutions. Managers at hospitals may use this survey to identify patients' views on the quality of treatment and their degree of satisfaction. According to, (Javed, Liu, Mahmoudi, & Nawaz., 2018), This research investigates the association between patient satisfaction and five characteristics of health care

service quality in both the public and private sectors of Pakistan's health care system. According to the Hurwicz criterion, people who get treatment from private healthcare institutions are more likely to be happy. The research advises that the SERVQUAL model be tailored for resource-strapped and impoverished nations, where people's perceptions of the quality of hospitals may be impacted by the cost of treatments. Any hospital's success depends on patient happiness. Customers who are happy with the hospital's services are urged to return. Their study seeks to establish a link between consumer happiness and hospital success.

A research from (Pandirkar, 2021) found a link between hospital services and customer happiness. Patients at private hospitals are happier than those in public hospitals. Peer to peer quality has a big influence on customer satisfaction and could help patients. Private hospitals want highly educated, competent, and experienced staff. Medical security is the most critical aspect of patient care. Customer satisfaction has been impacted by the doctor's consultation and treatment quality. Hospital administrative facilities also influence patient behavior. Patients love it when they can get maximal services at a low cost. Positive employee conduct enhances service quality and increases client happiness.

According to (Mehran Alijanzadeh, Rajaee, & Fard, 2016) Their study's major goal was to compare the quality of public and private health services in Qazvin, Iran. The findings indicated that in five categories, consumers' assessments of service quality in private hospitals were superior to those in state hospitals. The challenge of increasing public awareness of their entitlement to excellent services has raised the challenge of enhancing service quality

CHAPTER THREE

Materials and Method

3.1 Data Collection

3.1.1 Selection of Hospitals

The first step in our data collection process is the selection of government and private hospitals in the Munshiganj district. A purposive sampling technique will be employed to choose a representative sample of hospitals from each category. This selection will be based on factors such as hospital size, patient load, and geographical distribution within the district. A minimum of three government hospitals and three private hospitals will be included in the study to ensure diversity and comprehensiveness.

3.1.2 Data Sources

Data for this study will be collected from various sources:

Surveys: Structured survey was conducted among 95 patients who have received medical care in the selected government and private (3+3=6) hospitals. These surveys included questions related to their experiences, including accessibility, quality of care, cost, and overall satisfaction.

Hospital Records: Hospital records and documents, including patient wait times, billing information, and service offerings, was reviewed to gather objective data on the treatment systems.

Interviews: In-depth interviews was conducted with key hospital personnel, including healthcare providers, administrators, and staff, to gain insights into the operational aspects of both government and private hospitals.

Patient Feedback: We collected feedback from patients through various means and direct interactions with patients during the hospital visit.

3.2 Data Analysis

The specific statistical analysis methods used in our research project will depend on the data we have collected and the research questions we aim to answer. Below are some common statistical

analyses that might consider using in our project on the comparative analysis of treatment systems in government and private hospitals in Munshiganj:

1. Descriptive Statistics: Descriptive statistics, such as means, medians, standard deviations, and frequencies, used to summarize and present the characteristics of the collected data. For instance, we calculate the frequencies of respondent's gender, age, occupation.

2. Chi-Square Tests: Chi-square tests are appropriate for comparing categorical data. We used chi-square tests to determine significant differences between current medical institution type & previous medical institution type, hospitals are doing extra/unnecessary test? & current medical institution type and condition of disease in the treatment & previous medical institution type.

3. Regression Analysis: Regression analysis was employed to examine relationships between variables. For example, we used regression to investigate how factors like monthly family income of respondent & monthly family expenditure of respondent.

4. ANOVA (Analysis of Variance): Our study involves more than two groups or categories, ANOVA help us to compare means across multiple groups. We used ANOVA to assess whether there are significant differences in waiting times or other variables among different government and private hospitals in Munshiganj.

5. Correlation Analysis: Correlation analysis can help us explore the relationships between different variables. We used correlation analysis to understand how the quality of care provided in government hospitals correlates with patient satisfaction or other factors.

The specific statistical analyses used in our project determined by our research questions and the nature of our data. It's important to choose the appropriate statistical methods and ensure that our analysis was conducted rigorously to draw meaningful conclusions from our research.

3.2.1 Quantitative Data Analysis

Quantitative data obtained from surveys and hospital records was analyzed using statistical software. Descriptive statistics, such as means, medians, and standard deviations, was computed to summarize data related to accessibility, quality of care, and cost. Comparative analyses, such as t-tests and chi-square tests, will be conducted to identify statistically significant differences between government and private hospitals.

3.2.2 Qualitative Data Analysis

Qualitative data obtained from interviews and patient feedback was analyzed thematically. Transcripts from interviews was coded and categorized to identify recurring themes and patterns related to the strengths and weaknesses of each system. Patient feedback was reviewed for common trends and issues, providing additional context to the quantitative findings.

3.3 Ethical Considerations

This study adhered to ethical principles, and all data was collected with the informed consent of participants. Anonymity and confidentiality of patient data was maintained. Approval for this research was sought from relevant institutional review boards or ethical committees, and all ethical guidelines and regulations have strictly followed.

3.4 Limitations

It is essential to acknowledge potential limitations, such as the sample size, potential biases in self-reported data, and the geographic focus on Munshiganj. These limitations will be considered when interpreting the results.

This methodology guided our research efforts to provide a comprehensive comparative analysis of the treatment systems in government and private hospitals in Munshiganj. The data collection and analysis procedures helped us draw meaningful conclusions and policy recommendations based on our research findings.

3.5 Statistical Analysis

After assigning the variables, present author turned to the point of statistical analysis. It was one of the important parts of this study. Statistical analysis was carried out using the IBM SPSS 20 software. Descriptive statistics (frequency distributions) were first used to find out frequencies of all categorical variables. The outcomes were presented by bar diagram and pie chart. Chi- square test was used to find out the association of independent variables with dependent variable. Partial correlation was used to find out the association among monthly family income of respondent, monthly family expenditure of respondent & average monthly expenditure in medical field. Finally, linear regression model was used to find the monthly expenditure and average monthly of the respondents.

CHAPTER FOUR

Result and Discussion

4.1 Frequency Analysis & Graphical Representation

Frequency distribution is a table, graph, or other representation that shows the frequency of various outcomes in a dataset. It summarizes how often each value or category appears, making it easier to understand the distribution of data. In a frequency distribution, the data is grouped into intervals or categories, and the number of observations falling into each category is recorded. This helps identify patterns, central tendencies, and variations within the data.

Table-4.1.1: Frequency distribution of the respondent's Gender.

Gender of Respondent					
		Frequency	Percent	Valid Percent	Cumulative Percent
Valid	Male	53	55.8	55.8	55.8
	Female	42	44.2	44.2	100.0
	Total	95	100.0	100.0	

Comments: From the above table we observed that both male and female respondents are included in my research .Here 55.8 % respondents are male and 44.2% respondents are female .This indicates a slight majority of males in the surveyed population. The pie diagram of respondent's sex is given below:

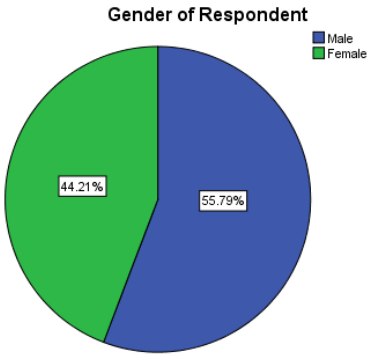

Fig.1: Pie diagram for respondent's Gender

Table-4.1.2: Frequency distribution of respondent's age.

Age Group					
		Frequency	Percent	Valid Percent	Cumulative Percent
	0-20	7	7.4	7.4	7.4
	21-40	63	66.3	66.3	73.7
	41-60	19	20.0	20.0	93.7
	60+	6	6.3	6.3	100.0
	Total	95	100.0	100.0	

Comments: This table displays the distribution of individuals in different age groups. The majority (66.3%) fall into the 21-40 year age range, followed by 20% in the 41-60 group. The 0-20 group represents 7.4%, and those aged 60 and above make up 6.3% of the total sample. The bar diagram of respondent's age group is given below:

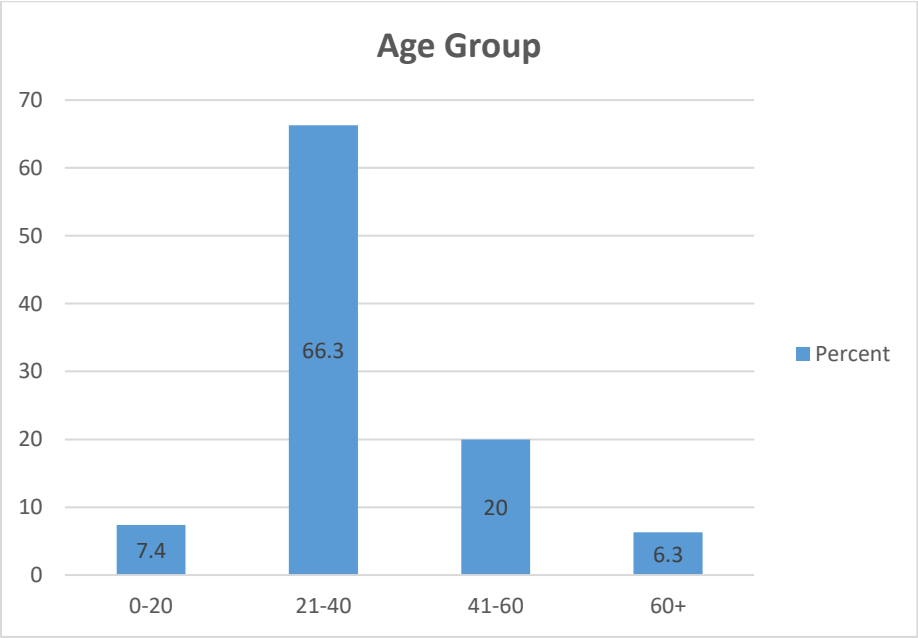

Fig 2: Bar diagram for respondent's Age group.

Table-4.1.3: Frequency distribution of respondent's Occupation.

Occupation of Respondent					
		Frequency	Percent	Valid Percent	Cumulative Percent
	Housewife	34	35.8	35.8	35.8
	Day labour	11	11.6	11.6	47.4
	Farmer	11	11.6	11.6	58.9
	Business	11	11.6	11.6	70.5
	Service	9	9.5	9.5	80.0
	Student	14	14.7	14.7	94.7
	Others	5	5.3	5.3	100.0
	Total	95	100.0	100.0	

Comments: From the above table we observed that 35.8% of the respondents are housewife, 11.6% are day laborer, 11.6% are farmer, 11.6% are businessman, 9.5% are job holder, 14.7% are student and 5.3% are others occupation. The data also shows that the majority of respondents are housewives, followed by a significant representation of day laborers, farmers, and individuals in business-related occupations. The pie diagram of respondent's occupation is given bellow:

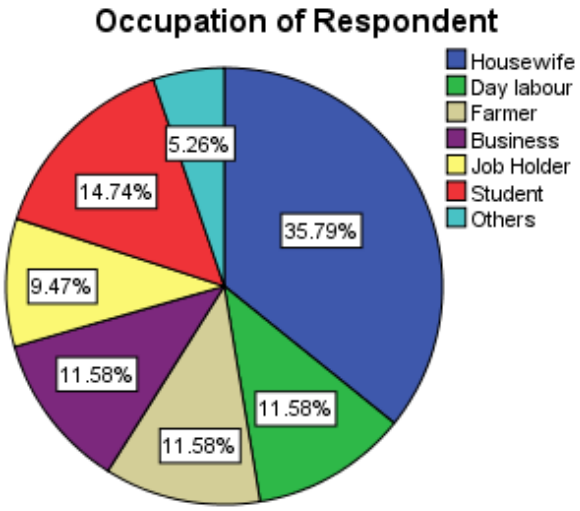

Fig 3: Pie diagram for respondent's occupation.

Table-4.1.4: Frequency distribution of respondent's Current type of Disease.

Current type of Illness Disease					
		Frequency	Percent	Valid Percent	Cumulative Percent
	General diseases, fever-cold	10	10.5	10.5	10.5
	Gastric/ulcer, thyroid	10	10.5	10.5	21.1
	Ear, Nose & Throat (ENT)	6	6.3	6.3	27.4
	Hypertension	2	2.1	2.1	29.5
	Heart disease	1	1.1	1.1	30.5
	Pregnancy	6	6.3	6.3	36.8
	Diabetics	2	2.1	2.1	38.9
	Asthma/Lung	3	3.2	3.2	42.1
	Others	18	18.9	18.9	61.1
	Two Diseases	26	27.4	27.4	88.4
	More Than Two Diseases	11	11.6	11.6	100.0
	Total	95	100.0	100.0	

Comments: The data reveals the types of illnesses or diseases among the respondents. The most common categories include "Two Diseases" and "Others." A substantial portion of respondents reported multiple health conditions. This information highlights the variety of health issues within the surveyed population, with a range of specific illnesses and combinations of diseases reported. The bar diagram of respondent's current type of disease is given bellow:

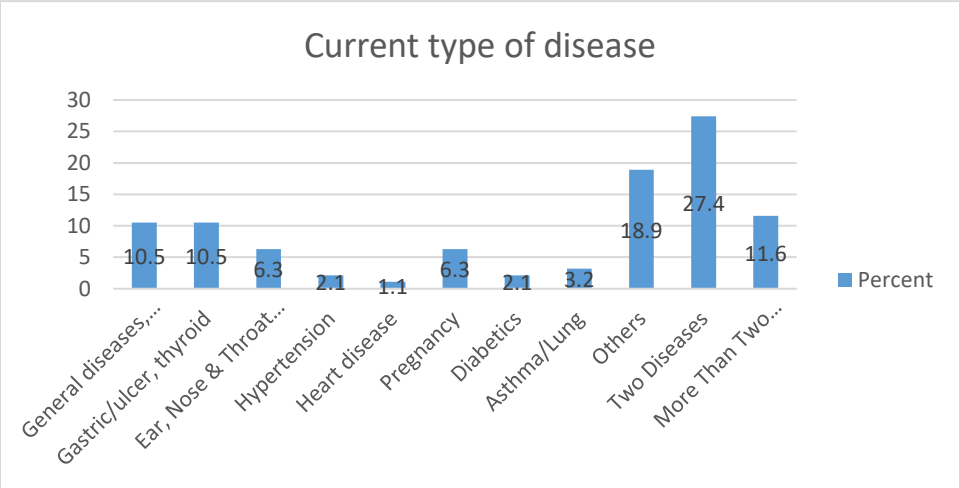

Fig 4: Bar diagram for respondent's current type of disease.

Table-4.1.5: Frequency distribution of respondent's average monthly expenditure in medical field.

Average monthly expenditure in medical field				
TK	Frequency	Percent	Valid Percent	Cumulative Percent
0-1000	13	13.7	13.7	13.7
1001-3000	35	36.8	36.8	50.5
3001-5000	30	31.6	31.6	82.1
5001-10000	12	12.6	12.6	94.7
10000+	5	5.3	5.3	100.0
Total	95	100.0	100.0	

Comments: This table illustrates the distribution of medical expenses. The largest group (36.8%) has expenses between 1001 and 3000. About 82.1% of the total sample has expenses up to 5000, and only 5.3% have expenses exceeding 10,000. The bar diagram of respondent's average monthly expenditure in medical field is given bellow:

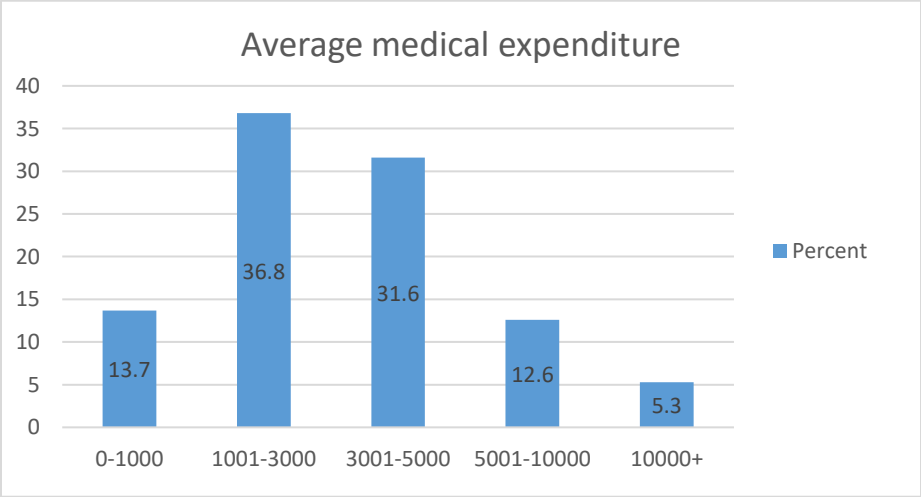

Fig 5: Bar diagram for respondent's average monthly expenditure in medical field.

Table-4.1.6: Frequency distribution of respondent's thinking of receiving treatment at this hospital.

Thinking of Receiving Treatment at This Hospital					
		Frequency	Percent	Valid Percent	Cumulative Percent
Valid	Low cost treatment	18	18.9	18.9	18.9
	Better Medical Care	59	62.1	62.1	81.1
	Treatment in less time	3	3.2	3.2	84.2
	Low Cost Treatment & Better Medical Care	15	15.8	15.8	100.0
	Total	95	100.0	100.0	

Comments: Most respondents (62.1%) prioritize "Better Medical Care" when considering treatment at the hospital. A significant portion (18.9%) values "Low cost treatment." Fewer respondents (3.2%) seek "Treatment in less time," and some (15.8%) value both "Low Cost Treatment" and "Better Medical Care." The pie diagram of respondent's thinking of receiving treatment at this hospital is given bellow:

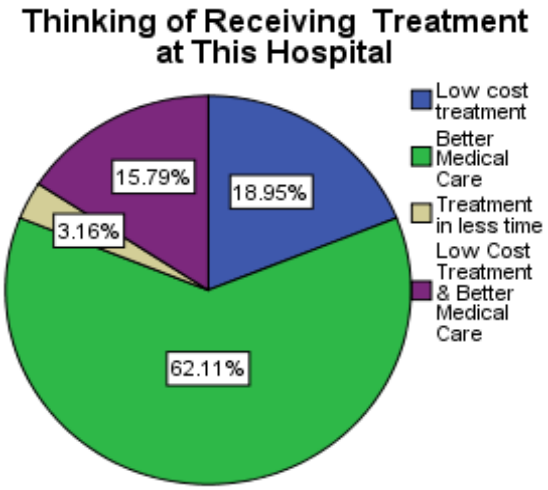

Fig 6: Pie diagram of respondent's thinking of receiving treatment at this hospital.

Table-4.1.7: Frequency distribution of respondent's last test name.

Last Test Name					
		Frequency	Percent	Valid Percent	Cumulative Percent
Valid	X ray	16	16.8	16.8	16.8
	Blood	27	28.4	28.4	45.3
	Others	24	25.3	25.3	70.5
	X ray & Others	10	10.5	10.5	81.1
	Blood & Others	18	18.9	18.9	100.0
	Total	95	100.0	100.0	

Comments: The data shows the last types of medical tests undergone by the respondents. "Blood" tests are the most common (28.4%), followed by "Others" (25.3%) and "X ray" tests (16.8%). Some respondents had a combination of tests, with "Blood & Others" being the most common combination. This information provides insights into the recent medical testing patterns among the surveyed population. The pie diagram of respondent's last test name is given bellow:

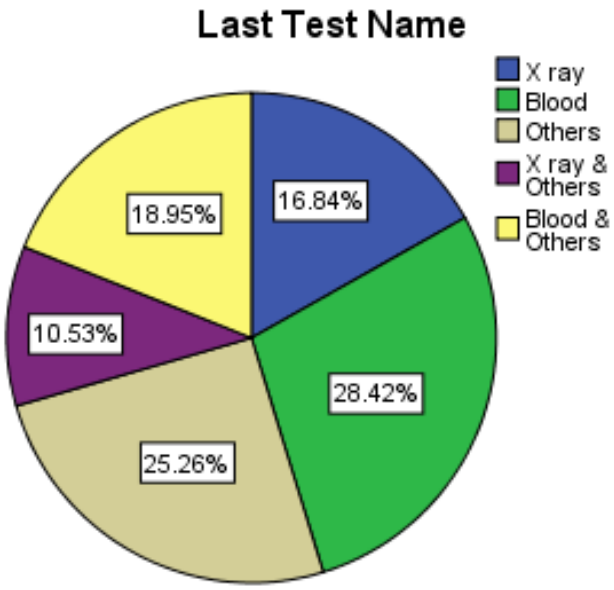

Fig 7: Pie diagram of respondent's last test name

Table-4.1.8: Frequency distribution of respondent's last Test Cost.

Test _Cost				
TK	Frequency	Percent	Valid Percent	Cumulative Percent
0-500	36	37.9	37.9	37.9
501-1000	23	24.2	24.2	62.1
1001-3000	26	27.4	27.4	89.5
3000+	10	10.5	10.5	100.0
Total	95	100.0	100.0	

Comments: The table shows the distribution of "Test Cost" data: About 37.9% of the observations have a test cost between 0 and 500.Approximately 24.2% have a test cost between 501 and 1000.Roughly 27.4% have a test cost between 1001 and 3000.Only 10.5% have a test cost of more than 3000.These percentages add up to 100% of the total 95 observations. The bar diagram of respondent's last test cost is given bellow:

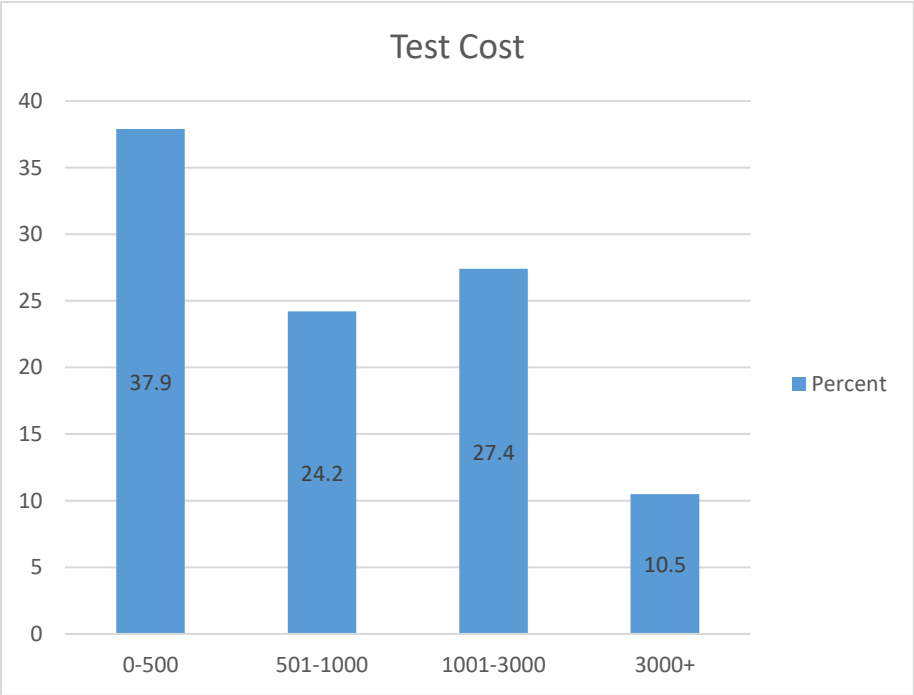

Fig 8: Bar diagram of respondent's last Test Cost.

Tabl3-4.1.9: Frequency distribution of respondent's condition of disease in the treatment.

Condition of disease in The Treatment					
		Frequency	Percent	Valid Percent	Cumulative Percent
Valid	Improvement	67	70.5	70.5	70.5
	Deterioration	15	15.8	15.8	86.3
	No change	13	13.7	13.7	100.0
	Total	95	100.0	100.0	

Comments: The data shows that the majority of respondents (70.5%) observed "Improvement" in their health condition during treatment, while a smaller portion (15.8%) reported "Deterioration." Some respondents (13.7%) noted "No change" in their condition, signifying stability. The pie diagram of respondent's condition of disease in the treatment is given bellow:

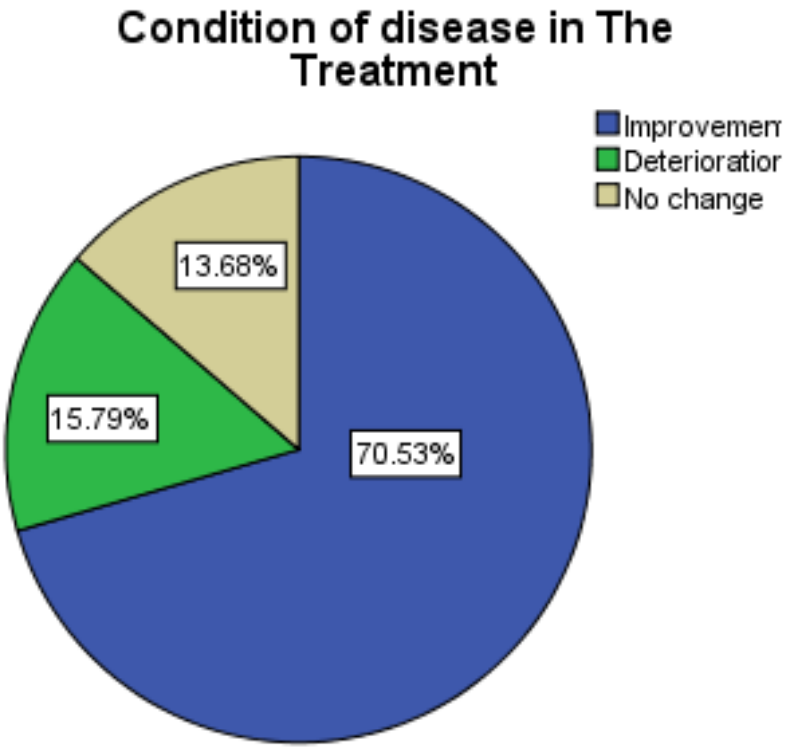

Fig 9: Pie diagram of respondent's condition of disease in the treatment.

Table-4.1.10: Frequency distribution of respondent's opinion about extra/unnecessary test of the hospitals.

Hospitals are doing extra/unnecessary test?					
		Frequency	Percent	Valid Percent	Cumulative Percent
Valid	Yes	17	17.9	17.9	17.9
	No	78	82.1	82.1	100.0
	Total	95	100.0	100.0	

Comments: In response to the question about hospitals conducting unnecessary tests: 17.9% of respondents believe hospitals are doing extra/unnecessary tests.82.1% of respondents do not believe hospitals are conducting extra/unnecessary tests. The pie chart of respondent's opinion about extra/unnecessary test of the hospitals is given bellow:

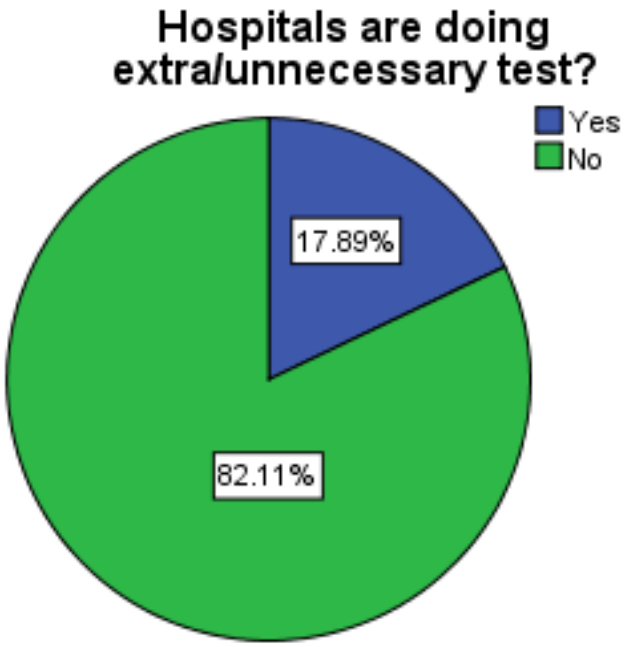

Fig 10: Pie diagram of respondent's opinion about extra/unnecessary test.

Table-4.1.11: Frequency distribution of services or facilities respondents want for satisfaction.

Services or Facilities Want for Satisfaction					
Service		Frequency	Percent	Valid Percent	Cumulative Percent
Valid	Further increase in quality of service	14	14.7	14.7	14.7
	Service in less time	2	2.1	2.1	16.8
	Necessary to reduce medical costs	10	10.5	10.5	27.4
	Improving utilization of staff	4	4.2	4.2	31.6
	Arrangement of transport/ambulance	4	4.2	4.2	35.8
	Further increase in quality of service & Improving utilization of staff"	4	4.2	4.2	40.0
	Further increase in quality of service & Service in less time"	2	2.1	2.1	42.1
	Further increase in quality of service & Medication administration	5	5.3	5.3	47.4
	Further increase in quality of service & Necessary to reduce medical costs	7	7.4	7.4	54.7
	More than two Facilities	43	45.3	45.3	100.0
	Total	95	100.0	100.0	

Comments: In this survey, individuals expressed various preferences for services and facilities to enhance their satisfaction: Many people (45.3%) wanted more than two facilities improved. Some common desires included better service quality (14.7%), reducing medical costs (10.5%), and improving staff utilization (4.2%).A smaller portion wanted faster services (2.1%) or improved transportation/ambulance services (4.2%).Some respondents had combined preferences, such as higher service quality with faster service (2.1%) or with reduced medical costs (7.4%).Overall, improving service quality was a prominent theme, and many respondents had more than one preference. The bar diagram of services or facilities respondents want for satisfaction is given bellow:

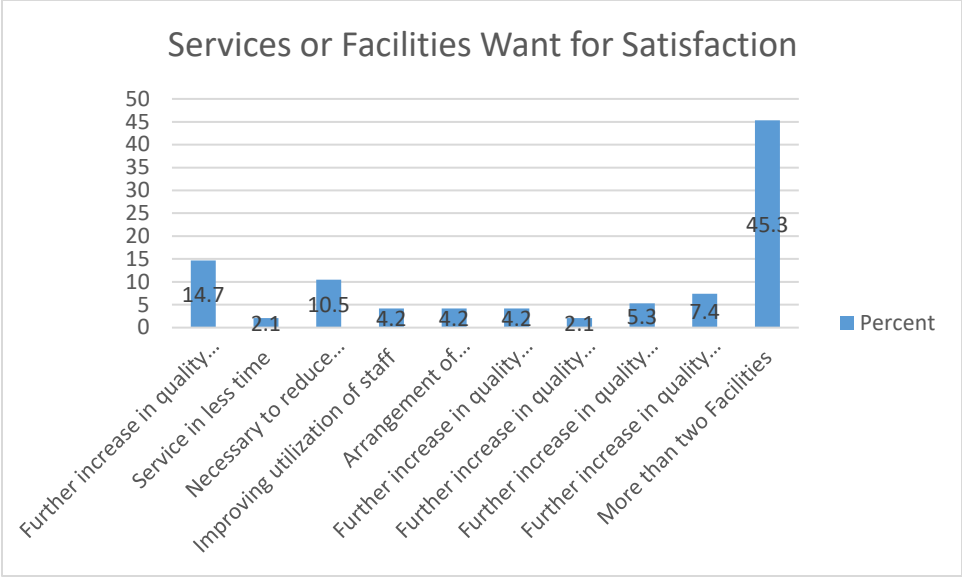

Fig.11: Bar diagram of services or facilities respondents want for satisfaction.

4.2 Contingency Analysis

We represent contingency analysis, which is designed to test any association between different phenomena. contingency analysis, if **'O'** denoted observation frequency and '**E'** denote expected frequency of a contingency table, then the chi-square statistic is,

Chi-Square statistic:

$$\chi^2 = \sum_i \sum_j \frac{O_{ij}^2}{E_{ij}} - N \sim \chi^2_{(r-1)(c-1)}$$

Which follows the chi-square distribution with (r-1)(c-1) degrees of freedom.

Where,

O_{ij} = The Observed number of data in (i, j) th cell

E_{ij} = the expected number of data in (i, j) th Cell

r=Number of rows, c=Number of columns, α =Level of significance.

Here,

H_0: There is no association between the given variables.

H_1: There is association between the given variables.

We know that the null hypothesis might be accepted at the 5% level of significance if the significance level is greater than 0.05; otherwise, the null hypothesis might be rejected.

4.2.1 Association between current medical institution type & previous medical institution type

Table-4.2.1: Cross tabulation for current medical institution type & previous medical institution type

			Previous Medical Institution Type			Total	Chi-square value	p-value
			Govt. hospital	private hospital	Both			
Current Medical Institution Type	Govt.	Count	32	9	4	45	23.779	0.0000
		% within Current Medical Institution Type	71.10%	20.00%	8.90%	100.00%		
	Private	Count	11	32	7	50		
		% within Current Medical Institution Type	22.00%	64.00%	14.00%	100.00%		
Total		Count	43	41	11	95		
		% within Current Medical Institution Type	45.30%	43.20%	11.60%	100.00%		

Comments: The chi-square test results indicate that there is a strong, statistically significant association between an individual's previous medical institution type and their current medical institution type. In other words, these two variables are not independent, and the choice of one is influenced by the other.

4.2.2 Association between hospitals are doing extra/unnecessary test? & current medical institution type

Table-4.2.2: Cross tabulation for hospitals are doing extra/unnecessary test? & current medical institution type

			Current Medical Institution Type		Total	Chi-square value	p-value
			Govt.	Private			
Hospitals are doing extra/unnecessary test?	Yes	Count	5	12	17	2.678	0.102
		% within Hospitals are doing extra/unnecessary test?	29.40%	70.60%	100.00%		
	No	Count	40	38	78		
		% within Hospitals are doing extra/unnecessary test?	51.30%	48.70%	100.00%		
Total		Count	45	50	95		
		% within Hospitals are doing extra/unnecessary test?	47.40%	52.60%	100.00%		

Comments: The chi-square test indicates that there is no statistically significant relationship between the type of hospital (government or private) and the perception of hospitals performing extra/unnecessary tests. Both types of hospitals have similar proportions of individuals who believe that extra/unnecessary tests are being conducted.

4.2.3 Association between condition of disease in the treatment & previous medical institution type

Table-4.2.3: Cross tabulation for condition of disease in the treatment & previous medical institution type

			Previous Medical Institution Type			Total	Chi-square value	P-value
			Govt. hospital	private hospital	Both			
Condition of disease in The Treatment	Improvement	Count	31	25	11	67	6.659	0.155
		% within Condition of disease in The Treatment	46.30%	37.30%	16.40%	100.00%		
	Deterioration	Count	7	8	0	15		
		% within Condition of disease in The Treatment	46.70%	53.30%	0.00%	100.00%		
	No change	Count	5	8	0	13		
		% within Condition of disease in The Treatment	38.50%	61.50%	0.00%	100.00%		
Total		Count	43	41	11	95		
		% within Condition of disease in The Treatment	45.30%	43.20%	11.60%	100.00%		

Comments: The chi-square test reveals that there is no statistically significant relationship determine between the type of previous medical institution (Govt. hospital or private hospital) and the condition of the disease during treatment (Improvement, Deterioration, No change). In other words, the previous medical institution type does not appear to be a significant factor in the condition of the disease during treatment.

4.3 Correlation

Pearson correlation is a statistical measure used to assess the strength and direction of the linear relationship between two continuous variables. It produces a value between -1 and 1, with 0 indicating no linear correlation. It is a fundamental tool in statistics for quantifying the degree of association between two variables. The formula for correlation between variables X and Y, is:

$$r_{xy} = \frac{\Sigma(x_i - \bar{x})(y_i - \bar{y})}{\sqrt{\Sigma(x_i - \bar{x})^2 \, \Sigma(y_i - \bar{y})^2}}$$

r_{xy} = correlation coefficient between x and y

x_i = the values of x within a sample

y_i = the values of y within a sample

$\bar{x}$ = the average of the values of x within a sample

$\bar{y}$ = the average of the values of y within a sample

4.3.1 Correlation among monthly family income of respondent, monthly family expenditure of respondent & average monthly expenditure in medical field.

Control Variables			Monthly Family Income of Respondent	Monthly Family Expenditure of Respondent	Average Monthly Expenditure in Medical Field
	Monthly Family Income of Respondent	Correlation	1	0.733	0.599
		Significance (2-tailed)	.	0	0
		df	0	93	93
	Monthly Family Expenditure of Respondent	Correlation	0.733	1	0.45
		Significance (2-tailed)	0	.	0
		df	93	0	93
	Average Monthly Expenditure in Medical Field	Correlation	0.599	0.45	1
		Significance (2-tailed)	0	0	.
		df	93	93	0
Average Monthly Expenditure in Medical Field	Monthly Family Income of Respondent	Correlation	1	0.648	
		Significance (2-tailed)	.	0	
		df	0	92	
	Monthly Family Expenditure of Respondent	Correlation	0.648	1	
		Significance (2-tailed)	0	.	
		df	92	0	

Comments: "Monthly Family Income" and "Monthly Family Expenditure" have a strong, statistically significant positive correlation, suggesting that as income increases, expenditure tends to rise. "Monthly Family Income" and "Average Monthly Expenditure in Medical Field" exhibit a moderate, statistically significant positive correlation, indicating that higher income is associated with higher medical expenditure. "Monthly Family Expenditure" and "Average Monthly Expenditure in Medical Field" show a weaker, statistically significant positive correlation, suggesting that higher family expenditure is linked to increased medical field expenditure.

4.4 Regression Analysis

Regression analysis is a statistical technique used to examine the relationship between one or more independent variables (also called predictors or explanatory variables) and a dependent variable (also known as the response or outcome variable).

The regression equation is a mathematical model that expresses the relationship between the independent and dependent variables. In simple linear regression, it can be represented as:

$$\mathbf{Y = \alpha + \beta X + \varepsilon}$$

Y is the dependent variable.

X is the independent variable.

α is the intercept (the value of Y when X is 0).

β is the slope (how much Y changes for a one-unit change in X).

ε is the error term, representing the unexplained variability in Y.

4.4.1. Regression model between monthly family income of respondent & monthly family expenditure of respondent.

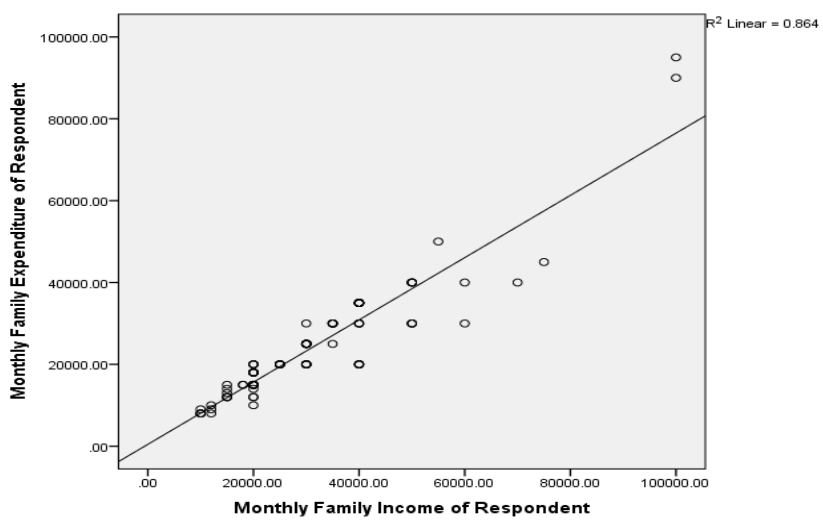

Fig 4.4.1: Scatter diagram of monthly family income & monthly family expenditure of respondent.

Model Summary				
Model	R	R Square	Adjusted R Square	Std. Error of the Estimate
1	.929[a]	.864	.862	5123.78409

ANOVA[a]						
Model		Sum of Squares	df	Mean Square	F	Sig.
1	Regression	15300038751.152	1	15300038751.152	582.788	.000[b]
	Residual	2415291036.082	92	26253163.436		
	Total	17715329787.234	93			

Coefficients[a]						
Model		Unstandardized Coefficients		Standardized Coefficients	t	Sig.
		B	Std. Error	Beta		
1	(Constant)	453.585	1126.037		.403	.688
	Monthly Family Income of Respondent	.760	.032	.929	24.141	.000

The regression equation is:

Y=453.585+0.760X

Here,

Y = Monthly Family Expenditure of Respondent

X = Monthly Family Income of Respondent

Comments: The regression analysis reveals a statistically significant positive relationship between Monthly Family Income and the dependent variable. For every one-unit increase in Monthly Family Income, the dependent variable is expected to increase by 0.760 units.

CHAPTER FIVE

Conclusion

Comparison

In this survey, the vast majority of patients visited private hospitals for their current treatment. A descriptive mean and difference analysis was performed on the data acquired by interviewing 95 patients from Munshiganj's govt. and private hospitals. In this paper, the main aim is to find the comparative treatment system delivered to patients by private and govt. hospitals in Bangladesh. To fulfill our study we conduct both male and female patients who received treatment from both government and private hospitals. 55.8 % respondents are male and 44.2% respondents are female. The majority of the respondents was aged from 21 to 40 who lived in Munshiganj.

To compare the government and private hospital's treatment system we consider various dimension. 71.10% of individuals who previously received treatment in government hospitals continue to receive treatment in government hospitals. 20.00% of individuals who previously received treatment in government hospitals have shifted to private hospitals. 8.90% of individuals who previously received treatment in government hospitals are currently receiving treatment from both government and private hospitals. 22.00% of individuals who previously received treatment in private hospitals have shifted to government hospitals. 64.00% of individuals who previously received treatment in private hospitals continue to receive treatment in private hospitals. 14.00% of individuals who previously received treatment in private hospitals are currently receiving treatment from both government and private hospitals. The information suggest that the patients tend to continue seeking treatment at the same type of hospital where they received treatment previously. Those who initially received treatment in government hospitals are more likely to continue treatment in government hospitals, and the same pattern applies to those initially treated in private hospitals.

On the dimension about the condition of disease during treatment, among individuals whose previous medical institution was a government hospital 46.30% experienced improvement in their condition during treatment, while 46.70% saw deterioration, and 38.50% reported no change in

their condition .Among individuals whose previous medical institution was a private hospital, 37.30% experienced improvement, 53.30% reported deterioration, and 61.50% observed no change in their condition. When considering all respondents together, regardless of their previous medical institution, 45.30% experienced improvement in their condition during treatment, 43.20% saw deterioration, and 11.60% reported no change in their condition. The data suggests that among individuals who previously received treatment in government hospitals, a slightly higher percentage experienced improvement in their condition during treatment compared to those who deteriorated. Conversely, among individuals with previous treatment in private hospitals, a slightly higher percentage experienced deterioration during treatment compared to those who improved.

About the perception of hospitals conducting extra/unnecessary tests, among individuals currently receiving treatment in government hospitals, 29.40% believe that hospitals are conducting extra/unnecessary tests. In contrast, 70.60% of them do not hold this perception. Among individuals currently receiving treatment in private hospitals, 48.70% believe that hospitals are conducting extra/unnecessary tests, while 51.30% do not share this perception. When considering both government and private hospitals together, 47.40% of respondents believe that hospitals are conducting extra/unnecessary tests, while 52.60% do not hold this perception. The data shows that a higher percentage of individuals currently receiving treatment in private hospitals (48.70%) perceive that hospitals are conducting extra/unnecessary tests compared to those in government hospitals (29.40%). While there is a difference in perception, it's important to note that neither group has a majority of respondents who believe that extra/unnecessary tests are being conducted.

In this study comparing government and private hospitals in Munshiganj, most patients opted for private hospitals. Patients tend to stay loyal to the same type of hospital they initially received treatment from, whether government or private. Treatment outcomes differed slightly between the two types of hospitals. Additionally, there was a perception gap regarding hospitals conducting extra tests, with a higher percentage of private hospital patients believing so. These findings provide valuable insights into patient behavior and healthcare practices, offering important implications for healthcare providers and policymakers in Bangladesh.

Recommendation

- The diversity of preferences among respondents highlights the multifaceted nature of patient expectations in healthcare. It's important for healthcare providers to consider and address a variety of factors to enhance patient satisfaction.
- The desire for more than two facilities to be improved suggests that patients have comprehensive expectations and value multiple aspects of their healthcare experience, including quality, efficiency, cost, and accessibility.
- Healthcare providers and policymakers should consider these preferences when designing and improving healthcare services to align with the expectations of the patient population.

Conclusion

In Munshiganj, individuals obtain healthcare from both the governmental and private sectors. The research conducted in Munshiganj, Bangladesh, presents several noteworthy findings. First and foremost, private hospitals emerge as the predominant choice for a significant majority of patients seeking medical treatment. A consistent trend is observed wherein patients tend to remain loyal to the same type of hospital they initially sought care from, be it a government or private facility.When examining treatment outcomes, the data indicates a slight disparity between government and private hospitals. Government hospitals exhibit a slightly higher percentage of cases with reported improvement in patients' conditions as compared to private hospitals. However, it's essential to note that both types of hospitals have room for improvement in delivering patient care effectively. Furthermore, the research highlights a noticeable perception gap concerning the belief that hospitals are conducting unnecessary tests. Interestingly, a higher percentage of patients from private hospitals express concerns about this practice. These findings hold valuable implications for healthcare providers and policymakers. They underscore the critical importance of comprehending and addressing patient preferences and perceptions. This understanding is crucial for enhancing the overall quality of healthcare services and ensuring patient satisfaction, making it a pivotal consideration in shaping the healthcare landscape of Bangladesh.

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পরিসংখ্যান বিভাগ, রাজশাহী বিশ্ববিদ্যালয়

[বিঃদ্রঃ- আপনার প্রদানকৃত তথ্য সম্পূর্ণ গোপন রাখা হবে এবং শুধুমাত্র গবেষণা কাজে ব্যবহার করা হবে]

মুন্সীগঞ্জের সরকারি এবং বেসরকারি হাসপাতালের চিকিৎসা ব্যবস্থার তুলনামূলক বিশ্লেষণ

(উত্তরে ☑ দিন,খালি বক্স গুলোতে ১ এর অধিক ☑ দিতে পারবেন)

১। নামঃ

২। লিঙ্গঃ নারী / পুরুষ

৩। বয়সঃ .....

৪। বৈবাহিক অবস্থাঃ বিবাহিত / অবিবাহিত

৫। পেশাঃ.....

৬। পারিবারিক মাসিক আয়ঃ .....

৭। পারিবারিক মাসিক ব্যয়ঃ .....

৮। বর্তমান আপনার রোগের/অসুস্থতার ধরণ,

☐ সাধারণ রোগ ,জ্বর-সর্দি ☐ গেষ্ট্রিক/আলসার,থাইরয়েড ☐ নাক,কান,গলা

☐ উচ্চ রক্তচাপ ☐ হৃদরোগ ☐ চোক্ষু ☐ গর্ভবতী

☐ ডায়বেটিকস ☐ শ্বাসকষ্ট / ফুসফুস ☐ অন্যান্য

৯। বর্তমানে আপনি কোন ধরণের প্রতিষ্ঠান থেকে চিকিৎসা সেবা নিচ্ছেন ?

a) সরকারি b) বেসরকারি c) উভয়ই

১০। এর আগে আপনি কোথায় চিকিৎসা নিয়েছেন?

a) সরকারি হাসপাতাল b) বেসরকারি হাসপাতাল c) উভয়ই

১১। চিকিৎসা ক্ষেত্রে গড়ে মাসিক ব্যয়ের পরিমানঃ .....

১২। চিকিৎসা ব্যয় বহণ করতে কখনো ধার-দেনা করতে হয়েছে কি? .

হ্যাঁ	না

১৩। যে হাসপাতালে আপনি চিকিৎসা নিচ্ছেন,সেই হাসপাতালে রোগীর তুলনায় পর্যাপ্ত সিটের ব্যবস্থা আছে কি ?

হ্যাঁ	না

১৪। কোন কোন দিক চিন্তা করে আপনি এই হাসপাতালে চিকিৎসা গ্রহণ করতে আসছেন?

a) কম খরচে চিকিৎসা b) উন্নত চিকিৎসা সেবা c) কম সময়ে চিকিৎসা d) a) & b)

১৫। ডাক্তাররা কি পর্যাপ্ত সময় নিয়ে প্রতিটি রোগী পর্যবেক্ষণ করেণ ?

হ্যাঁ	না

১৬। টেষ্টের জন্য এখানে প্রয়োজনীয় সকল ব্যবস্থা কী?

হ্যাঁ	না

১৬ (i) যদি না ,হয় তাহলে কোথায় করাণ?

a) সরকারি হাসপাতাল b) বেসরকারি হাসপাতাল c) উভয়ই

১৭। আপনি সর্বশেষ কি কি টেষ্ট করিয়েছেন ?

টেস্টের নাম	খরচ
১।	
২।	
৩।	

১৮। আপনি কি হাসপাতালে চিকিৎসার জন্য রাত্রীযাপন করেছেন ?

হ্যাঁ	না

(i) হ্যাঁ ,হলে কতো দিন /বার ? .....

১৯। আপনি কি চিকিৎসার জন্য কোনো হাসপাতাল পরিবর্তন করেছেন ?

হ্যাঁ	না

১৯ (i) হ্যাঁ , হলে কোন হাসপাতাল থেকে কোন হাসপাতালে গেছেন?

a) সরকারি-সরকারি (b সরকারি-বেসরকারি c) বেসরকারি-সরকারি d) বেসরকারি-বেসরকারি

১৯ (ii) এই পরিবর্তনের কারণ কি ?

- ☐ অর্থনৈতিক কোনো সুবিধা
- ☐ ভালো সেবা
- ☐ কম সময়ে চিকিৎসা
- ☐ স্বাস্থ্যকর পরিবেশ

২০ । চিকিৎসায় রোগের কি ধরণের পরিবর্তন হয়েছে/হচ্ছে ?

a) উন্নতি b) অবনতি c) পরিবর্তন হয়নি

২১। আপনি কি মনে করেন হাসপাতালগুলোতে অতিরিক্ত / অপ্রয়োজনীয় পরীক্ষা করা হয় ?

হ্যাঁ	না

২২। কি ধরণের সুবিধা বা সেবা আপনি হাসপাতাল থেকে চান , যা আপনার সন্তুষ্টি বৃদ্ধি করবে ?

- ☐ সেবার মান আরো বৃদ্ধি
- ☐ চিকিৎসা খরচ কমানো প্রয়োজন
- ☐ কম সময়ে সেবা প্রদান
- ☐ ষ্টাফদের ব্যবহারের উন্নতি
- ☐ পরিস্কার-পরিচ্ছন্নতা বৃদ্ধি
- ☐ পরিবহন/এম্বোলেন্সের ব্যবস্থা
- ☐ ভ্লোমানের খাবার
- ☐ ঔষধের ব্যবস্থা

২৩। হাসপাতালে ভর্তি ও রিলিজের ক্ষেত্রে কোনো জটিলতা ছিলো কি ?

হ্যাঁ	না

২৩ (i) হ্যাঁ,হলে কি ধরণের জটিলতা ছিলো ?

☐ সঠিক দিক নির্দেশনার অভাব ☐ সঠিক তথ্যের অভাব ☐ ফরমালিটিতে সময় বেশি লাগা